CLINICAL TRIAL FAILURES · DISCONTINUATION RATE · ONCOLOGY PHASE 2,3 · STARTS 2015–2024

FGFR: 5 molecules stopped early, all small molecules

6 stopped trials, but 5 molecules — all small molecules: Regorafenib, Fgfr Inhibitor Debio 1347, Infigratinib, Nintedanib and 1 more. Against 5.2% for all oncology trials with a resolved drug target and 4.5% across all oncology Phase 2,3 trials in the same window. The rate counts trials that stopped early for efficacy, safety or benefit–risk reasons. Trials that ran to completion and missed their primary endpoint are counted separately below, never in the rate.

3.6%
6 of 167 closed trials (95% CI 1.7%–7.6%)

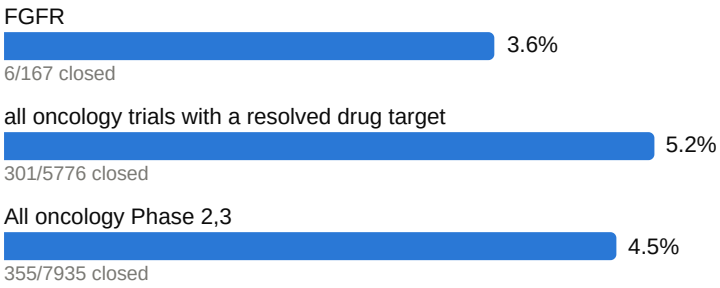
3 / 0 / 1
efficacy / safety / both (adds to 6)

47.7%
of 350 trials have closed · 183 still open or unresolved

1.7%
stopped for a biological reason within 3 years of starting (95% CI 0.6%–4.4%; 161 still at risk; 1.5% area-wide)

1 of 2 **Completed and missed the primary endpoint.** Of 103 completed trials in this segment, 2 posted a primary result on ClinicalTrials.gov that can be read — the sponsor's own comparison held to the sponsor's own threshold, or its statement that the endpoint was missed: 1 missed and 1 met. Counted separately and never in the rate above; a floor, not a rate. 47 of the 95 completed trials that finished more than a year ago have posted no results at all — a gap in what can be known, not a sign of failure.

How this compares



What is in this class: 212 distinct experimental drugs across 350 trials (small molecule 340, antibody 126, unknown 44, adc 7). Most tested: REGORAFENIB (108), SURUFATINIB (67), PEMIGATINIB (36), PAZOPANIB (26), NIVOLUMAB (23). A class groups drugs by what they act on; check that the grouping is one you would make before reading the rate as a property of the mechanism.

How much does this rest on one decision? The 6 stops came from 6 sponsor-asset programmes across 6 sponsors; the largest (University of Texas Southwestern Medical Center) contributed 1; removing that programme's trials from both sides leaves 3.0%. Screened alongside 85 other segments, the rate is within what screening that many classes could produce (q=1); the molecules and trials that failed here are the record either way. A further 19 closed trials here were terminated with no cause recorded in the registry; if every one of them were biological the rate would be 15.0%. Read it as a screen worth checking against the underlying trials.

Bars show the share of closed trials stopped for biological reasons. Confidence intervals overlap where sample sizes are small — read the counts, not just the bars.

By start cohort

Trials started	Stops	Closed	Rate	95% CI
2015–2017	1	75	1.3%	0.2%–7.2%
2018–2020	3	53	5.7%	1.9%–15.4%
2021–2024	2	39	5.1%	1.4%–16.9%

Where the stops sit

Efficacy / futility: 3, Benefit–risk: 2, Efficacy and safety: 1.
Sponsors with most stops: Great Novel Therapeutics Biotech & Medicals Corporation (1), Incyte (1), UNICANCER (1), University of Texas Southwestern Medical Center (1), Debiopharm International SA (1), National Institute of Dental and Craniofacial Research (NIDCR) (1).

The stopped trials

Trial	Ph	Sponsor	Experimental drugs	Stopped	Type	Registry stop reason
NCT05770882	2	Great Novel Therapeutics Biotech & Medicals Corporation	TUCIDINOSTAT, REGORAFENIB	2025-01	Benefit–risk	The overall profile does not support development for Hepatocellular Carcinoma

Trial	Ph	Sponsor	Experimental drugs	Stopped	Type	Registry stop reason
NCT05267106	2	Incyte INCY	PEMIGATINIB	2024-12	Efficacy	Recruitment ceased after a pre-planned futility interim analysis indicated a low probability to confer a clinically meaningful improvement in objective response when compared to currently available therapies. There were no safety related concerns.
NCT03722108	2	UNICANCER	REGORAFENIB, IRINOTECAN	2022-05	Benefit–risk	Unfavorable benefit-risk balance in the experimental arm following the IDMC for intermediate efficacy and safety analysis
NCT02902484	2	University of Texas Southwestern Medical Center	NINTEDANIB	2021-10	Efficacy	The study was terminated due to futility and lack of future funding
NCT03834220	2	Debiopharm International SA	FGFR INHIBITOR DEBIO 1347	2020-12	Efficacy	Due to lower antitumor activity than expected
NCT03510455	2	National Institute of Dental and Craniofacial Research (NIDCR)	INFIGRATINIB	2020-02	Efficacy + safety	Study was stopped early due to a greater than expected incidence of ocular AEs and analysis of the data from the first 4 subjects indicating that the likelihood of permanent remission with this therapy was low.

Method
Denominator: ClinicalTrials.gov interventional Phase 2,3 oncology trials started 2015–2024 that have closed (completed or terminated). Numerator: terminated trials whose registry stop reason is classified as biological (efficacy, safety or benefit–risk) by Classification V2 — held-out precision 95.5%, recall 95.3% (n=600). Drugs are linked to ChEMBL and the NCI Thesaurus; 76% of industry oncology trials in this window carry a resolved drug target, and a trial without one cannot enter a mechanism class. Intervals are Wilson 95%.

Limits
Not a failure rate: completed trials that missed their endpoints are not in the rate. They are counted separately where the sponsor posted a primary result that can be read — most did not, so that count is a floor — and programmes discontinued after a completed trial do not appear. Stop reasons are sponsor-reported. This is a closed-trial proportion, not a time-to-event analysis: only 47.7% of trials in this segment have closed, and a trial that stops early enters the denominator sooner than one that runs to completion, which can inflate the rate in immature segments. Recent cohorts have fewer closed trials, so their rates are less stable. Research signals, not clinical or investment advice.

The whole cohort, trial by trial. The diligence report names every trial behind these figures — the ones still running and the ones that completed and missed their primary endpoint included — and places your molecule against each one that failed. Free during the beta, in exchange for a work email, delivered immediately: **clinicaltrialfailures.com/asset-check**

Sources: ClinicalTrials.gov (NLM); ChEMBL (EMBL-EBI, CC BY-SA 3.0); NCI Thesaurus (NCI); RxNorm/RxClass (NLM); SEC EDGAR. Classification, linkage and rates by Clinical Trial Failures. Rebuilt weekly; this brief covers trials started 2015–2024.